

Office of Enforcement and Removal Operations  
ICE Health Service Corps

U.S. Department of Homeland Security  
500 12th Street, SW  
Washington, D.C. 20536



U.S. Immigration  
and Customs  
Enforcement

June 17, 2022

MEMORANDUM FOR:

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Assistant Director  
ICE Health Service Corps

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THROUGH:

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Deputy Assistant Director of Clinical Services/Medical Director  
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CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C  
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FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) RN  
Senior Investigator  
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings  
Felipe MONTES, (b)(6),(b)(7)(C)

**Executive Summary:**

On January 30, 2021, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Felipe MONTES, (b)(6),(b)(7)(C). During his detention period, ICE housed Mr. MONTES, a 57-year-old Mexican male at Stewart Detention Center (SDC) in Lumpkin, Georgia (GA), from December 28, 2020 to January 30, 2021, the date of his death.

On March 4, 2019, prior to Mr. MONTES' ICE custody and detention, ICE Enforcement and Removal Operations (ERO) Atlanta encountered Mr. MONTES at the Wheeler County Sheriff's Office (WCSO) in Alamo, GA, where he was serving felony convictions of sexual battery and

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criminal trespass. ERO served him an Immigration Detainer, Notice to Action, form I-247A. On December 23, 2020, ERO Atlanta encountered and conducted a custodial interview with Mr. MONTES at WCSO for his prior felony convictions of sexual battery and criminal trespass. On December 28, 2020, WCSO released Mr. MONTES to ERO Atlanta. On this same date, ERO Atlanta served Mr. MONTES a Notice to Appear (NTA), Form I-862, charging inadmissibility pursuant to Section 212(a)(6)(A)(i) of the Immigration and Nationality Act, as amended, which reads you are an alien present in the United States without being admitted or paroled, or who arrived in the United States at any time or place other than as designated by the Attorney General. On this same date, ERO Atlanta transferred Mr. MONTES to SDC for housing, where he remained for the duration of his ICE custody.

On December 26, 2020, during his intake screening, Mr. MONTES denied any past or current medical problems and denied coronavirus disease 2019 (COVID-19) related symptoms.

On January 30, 2021, at approximately 12:27 a.m., Mr. MONTES went into cardiopulmonary arrest (no heartbeat, and no spontaneous breathing) at Piedmont-Columbus Regional Hospital (PCRH). The medical staff initiated advanced life support measures; however, their efforts were unsuccessful. A PCRH MD pronounced Mr. MONTES deceased at 12:31 a.m. with a preliminary cause of death as cardiopulmonary arrest, secondary to complications of COVID-19.

### **Mortality Findings:**

As a result of Mr. MONTES' death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. MONTES' medical care at SDC was not provided within the safe limits of practice. Specifically, the MRC determined SDC staff did not follow their own policies regarding COVID-19 to prevent exposure to the virus, and their emergency protocols are outdated.

The MRC convened on May 20, 2021, and consisted of the following IHSC members: Captain (b)(6),(b)(7)(C),(b)(7)(F) Central Regional Nurse Manager; Dr. (b)(6),(b)(7)(C),(b)(7)(F) Clinical Director; and Mr. (b)(6),(b)(7)(C),(b)(7)(F) physician assistant.

The following summary of events was based on: 1) Mr. MONTES' SDC medical records and emergency medical services (EMS) report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. MONTES' SDC (b)(6),(b)(7)(C) detention file; 5) an on-site

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review and staff interviews conducted by factfinder, Commander (b)(6),(b)(7)(C),(b)(7)(F) at SDC on March 9-11, 2021; and 6) applicable SDC policies and procedures, and ICE Detention Standards.

### ***Detailed Medical Review***

On December 26, 2020, a registered nurse (RN) completed Mr. MONTES' pre-screening and intake screening, documented no history or current medical problems, and no COVID-19 related symptoms. The RN performed Mr. MONTES' COVID-19 test, placed him in quarantine intake per SDC's COVID-19 protocols, and ordered daily symptom screening checks. The medical record did not include any documentary evidence to confirm completion of Mr. MONTES' daily symptom screening checks.

On December 27, 2020, a behavioral health provider completed Mr. MONTES' routine comprehensive mental health screening, documented no abnormal findings, and no additional referrals.

On December 29, 2020, Mr. MONTES received a chest X-ray to rule out tuberculosis (a bacteria that affects the lungs). The results were negative.

On January 1, 2021, Mr. MONTES' COVID-19 test result showed negative results, and Mr. MONTES was placed in general population.

On January 4, 2021, an advanced practice provider (APP) completed Mr. MONTES' initial physical exam, documented normal findings, except for dry eyes, a right eye pterygium (a fleshy tissue growth on the conjunctiva of the eye), and provided Mr. MONTES with artificial tear solution to use, as needed. Mr. MONTES declined administration of the flu vaccination, and the APP scheduled his annual physical exam on December 8, 2021.

On January 10, 2021, at approximately 5:17 p.m., a custody officer (CO) assigned to Mr. MONTES' dormitory noted Mr. MONTES having difficulty breathing and called a medical emergency. Upon arrival to the scene, the medical staff transferred Mr. MONTES to the medical department, via stretcher.

- At 5:25 p.m., an RN evaluated Mr. MONTES, noted Mr. MONTES' low oxygen saturation [(SpO2): fraction of oxygen saturated hemoglobin in the blood] ranging from 74 – 78 percent (%) (normal oxygen saturation 95 – 100%) on room air, and contacted the on-call provider regarding his abnormal results. The APP gave the RN a verbal order to administer supplemental oxygen, and to contact emergency medical services (EMS) to transfer Mr. MONTES to the local emergency department (ED) for further evaluation.
- At approximately 6:05 p.m., EMS departed SDC and transported Mr. MONTES to the PCRH ED in Columbus, GA.

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- PCRH diagnosed Mr. MONTES with pneumonia related to COVID-19, hypoxia (deficiency in the amount of oxygen reaching the tissues), performed a COVID-19 test that showed positive test results, and admitted him into PCRH's inpatient medical unit.

On January 15, 2021, Mr. MONTES' condition improved, and PCRH considered discharging him back to SDC on January 18, 2021.

On January 19, 2021, Mr. MONTES' condition worsened, and he required placement on a continuous positive airway pressure device [(CPAP); used to keep the lungs expanded for better oxygen absorption during breathing]. Later in the day, Mr. MONTES removed his CPAP mask and his SpO2 decreased to 40%, requiring subsequent intubation (the process of placing a tube in the trachea for mechanical ventilation support), and transfer to the intensive care unit. The MD diagnosed Mr. MONTES with acute hypoxic respiratory failure secondary to COVID-19 infection.

From January 20 to 28, 2021, Mr. MONTES' condition continued to deteriorate.

On January 29, 2021, Mr. MONTES' vital signs showed critically low results of blood pressure – 49/41 millimeters of mercury [(mmHg); normal range is 90/60 – 120/80 mmHg], heart rate – 117 beats per minute [(bpm); normal range is 60 – 100 bpm], temperature – 96.9 degrees (°) Fahrenheit (F) (normal range is 97.0 – 99.0°F), and SpO2 of 48%, with the mechanical ventilatory support.

On January 30, 2021, at approximately 12:27 a.m., Mr. MONTES went into cardiopulmonary arrest (no heartbeat and no spontaneous breathing). PCRH medical staff initiated advanced life support measures; however, their efforts were unsuccessful.

At approximately 12:31 a.m., a PCRH MD pronounced Mr. MONTES deceased.

The final autopsy report is pending.

Detention Standards used for this review: ICE Performance-Based National Detention Standards (PBNDS), 2011 (revised December 2016), and the National Commission on Correctional Health Care (NCCHC) Standards, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

### ***Strengths***

Strengths identified during this investigation were: SDC medical staff responded within four minutes of SDC custody staff calling for emergency response, providing Mr. MONTES needed emergency medical care, including notifying EMS and sending him to the hospital.

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## **Weaknesses**

### **1. Documentation**

- On January 10, 2021, at 1:46 p.m. and 2:31 p.m., an SDC CO called the SDC medical clinic and reported Mr. MONTES had difficulty breathing. The SDC medical CO did not document receiving the phone calls, or that they had notified medical staff of the phone calls into the assigned logbook for record keeping.

SDC medical post orders (PO) did not include instructions or guidance for officers regarding appropriate documentation related to incoming phone calls from the dormitory requiring urgent and emergent attention.

- On January 10, 2021, two SDC medical staff responded to Mr. MONTES' medical emergency but did not document their response, assessment, or treatment.

*Applicable standards of care for this finding:*

- SDC PO: SDC-PO-111, Medical Post Order.
- PBNDS 2011: 4.3, Medical Care; section B.B. Medical Records.
- NCCHC 2018 standards, J-A-08, Health Records.

### **2. Care**

- Between December 30, 2020 and January 1, 2021, three detainees housed in Mr. MONTES' dormitory were diagnosed with COVID-19. Prior to the three detainees complaining of COVID-19 symptoms, the infected detainees comingled with Mr. MONTES and other detainees in the dormitory common area. Detainees were not required to wear face masks, and social distancing was not enforced while detainees comingled in the common area. Once the three detainees were identified as COVID-19 positive, SDC did not start the 14-day quarantine period over for the exposed detainees.

SDC did not follow 14-day intake quarantine procedures by allowing different cohorts (detainees who arrived at the facility more than 48 hours apart of one another) to comingle in the dormitory common area without wearing masks and social distancing. SDC did not restart the 14-day quarantine period for those exposed to COVID-19 positive detainees as per SDC local operating procedures (LOP), CoreCivic COVID-19 Pandemic Outbreak Operations and Management Control Plan, ICE ERO COVID-19 Pandemic Response Requirements (PRR), CDC Guidelines for Correctional Facilities, and *Frailhat v. ICE*.

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- On January 4, 2021, the clinical director identified Mr. MONTES as a high risk/vulnerable population member nine days after his arrival.

SDC did not identify and monitor Mr. MONTES as a high risk/vulnerable individual and subclass member within five days of arrival, per ICE ERO COVID-19 PRR and *Fraihat v. ICE*.

- On January 7, 2021, SDC medical staff released Mr. MONTES from 14-day intake quarantine. SDC custody staff housed Mr. MONTES in a high risk/vulnerable population dormitory.

SDC medical and custody staff did not follow CoreCivic plans, CDC guidelines, SDC's own local operating procedure, and ICE ERO COVID-19 PRR, and protect high risk/vulnerable detainees from an individual who had recent exposure to COVID-19.

- On January 9, 2021, Mr. MONTES reported having a sore throat during his COVID-19 symptom screening. An SDC RN reviewed Mr. MONTES' symptom screening form but did not assess or refer Mr. MONTES to a qualified medical health care provider for evaluation.

SDC medical staff did not follow CDC guidelines, their own local operating procedure, and ICE ERO COVID-19 PRR related to positive COVID-19 symptom screening.

*Applicable standards of care for this finding:*

- CoreCivic Policy: 13-34, Medical Emergency Response, section 4. Procedures, B. Training, 1. a. ii., v., and x.
- CoreCivic COVID-19 Pandemic Outbreak Operations and Management Control Plan: 13-91AA, Sections: V. Communication, F., G., J.; VII. High Risk Inmate/Detainee Population Strategies, A., E.; VIII. Quarantine, A., B., C., E., and I.
- CDC Guidelines for Correctional Facilities: <https://www.cdc.gov/coronavirus/2019-ncov/community/correction-detention/index.html>.
- ERO COVID-19 Pandemic Response Requirements Version 5.0, p. 8, 10.
- *Fraihat v. ICE*, 445 F. Supp. 3d 709 (C.D. Cal. 2020).
- SDC LOP: High Risk/Vulnerable Population Temperature Check and COVID-19 Symptoms Screening.
- SDC LOP: Medical Coronavirus Plan, section 5. Protect the Uninfected, a.
- SDC PO: CCA-PO-16, Housing Unit Officer Post Order.
- SDC PO: FAC-PO-00, General Orders, Section M. Injuries/Illnesses, 2. b.

### 3. Communication

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- On January 10, 2021, at 1:46 p.m. and 2:31 p.m., an SDC dormitory CO called the SDC medical CO and reported Mr. MONTES having difficulty breathing. The SDC dormitory CO did not send Mr. MONTES to the medical clinic, and the medical CO did not document receipt of any incoming phone calls. Per an interview with the health services administrator, SDC custody and medical staff do not always communicate, and medical staff did not receive the requests for Mr. MONTES to come to medical.

The SDC medical post order does not instruct custody staff on how to proceed when there is an urgent medical need request from a dormitory officer.

*Applicable standards of care for this finding:*

- CoreCivic Policy: 13-34., Medical Emergency Response, section 4. Procedures, B. Training, 1. a. ii., v., and x.
- SDC PO: SDC-PO-111, Medical Post Order.

#### **4. Emergency Response**

- On January 10, 2021, at 1:46 p.m. and 2:31 p.m., an SDC CO called the SDC medical clinic and reported Mr. MONTES having difficulty breathing. At 5:17 p.m., the SDC custody officer called a medical emergency for Mr. MONTES having difficulty breathing.

The SDC CO did not follow CoreCivic policy and SDC LOPs regarding identifying and responding to medical emergencies. The SDC CO did not notify a supervisor regarding Mr. MONTES not receiving treatment for difficulty breathing after the second phone call. SDC's LOP for housing unit officer post orders did not provide instruction to staff on how to manage urgent medical requests.

- SDC's LOP for emergency medical procedures is outdated and prioritizes transporting the detainee to the medical clinic and notifying the shift supervisor, before medically assessing and stabilizing the detainee and calling 911.

*Applicable standards of care for this finding:*

- CoreCivic Policy: 13-34., Medical Emergency Response, section 4. Procedures, B. Training, 1. a. ii., v., and x.
- NCCHC 2018: J-B-08, Patient Safety.
- NCCHC 2018: J-D-07, Emergency Services and Response Plan.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. Emergency Medical Services and First Aid.

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- SDC LOP: 13-57B, Emergency Medical Procedures.
- SDC PO: CCA-PO-16, Housing Unit Officer Post Order.

### ***Recommendations***

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, SDC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.